

PARTICIPATING PROVIDER AGREEMENT

Managed Care Network Participation & Reimbursement

Payer / Plan:	Pioneer Valley Medicaid MCO
Plan Type:	Medicaid Managed Care
Provider Organization:	Northwind Health
Provider Tax ID:	35-1899042
Contract Number:	NWH-2024-008
Reimbursement Methodology:	Percent of Medicaid fee schedule
Effective Date:	January 01, 2024
Termination Date:	December 31, 2025
Renewal / Notice Date:	September 01, 2025

This Participating Provider Agreement (the “Agreement”) is entered into by and between **Pioneer Valley Medicaid MCO** (“Payer”) and **Northwind Health** (“Provider”), effective as of January 01, 2024.

This cover page is a summary for administrative convenience only. In the event of any conflict between this cover page and the body of the Agreement, the body of the Agreement controls. The Exhibits, Appendices, and any duly executed Amendments are incorporated herein by reference.

RECITALS

WHEREAS, Pioneer Valley Medicaid MCO ("Payer") arranges for the provision of health care services to individuals enrolled in one or more Medicaid Managed Care benefit plans for which Payer provides or administers coverage (each, a "Member"); and

WHEREAS, Northwind Health ("Provider") is a duly licensed health system operating the hospitals, ambulatory facilities, and professional practices identified in Appendix 1, and holds federal Tax Identification Number 35-1899042; and

WHEREAS, Provider desires to participate in Payer's provider network and to render Covered Services to Members, and Payer desires to include Provider in its network on the terms set forth herein; and

WHEREAS, the parties intend that this Agreement comply with all applicable federal and state laws governing the delivery and reimbursement of health care services, including the privacy and security standards of the Health Insurance Portability and Accountability Act of 1996 (HIPAA);

NOW, THEREFORE, in consideration of the mutual covenants and promises set forth herein, and other good and valuable consideration, the receipt and sufficiency of which are hereby acknowledged, the parties agree as follows:

ARTICLE 1. DEFINITIONS

As used in this Agreement, the following capitalized terms shall have the meanings set forth below. Terms not defined herein shall have the meaning ascribed to them under applicable law or, where applicable, the relevant Plan documents.

1.1 "Clean Claim" means a claim submitted on the applicable CMS-1450 (UB-04) or CMS-1500 form, or its electronic 837 equivalent, that requires no further information, documentation, or substantiation in order to be adjudicated and that is not subject to a pending investigation of fraud or abuse.

1.2 "Covered Services" means those medically necessary health care services and supplies that are benefits under the applicable Plan and that are rendered to a Member by Provider during the term of this Agreement.

1.3 "Contracted Rate" means the amount payable to Provider for a Covered Service as set forth in the Fee Schedule in Article 11 and Exhibits A and B, as the same may be amended from time to time in accordance with Article 18.

1.4 "Medical Necessity" or "Medically Necessary" means health care services that a prudent physician would provide for the purpose of preventing, evaluating, diagnosing, or treating an illness, injury, or disease, consistent with generally accepted standards of medical practice and not primarily for the convenience of the Member or Provider.

1.5 "Member" means an individual who is enrolled in and eligible for benefits under a Plan administered or insured by Payer on the date a Covered Service is rendered.

1.6 "Plan" means a health benefit plan offered, insured, or administered by Payer under the Medicaid Managed Care product line to which this Agreement applies.

1.7 "Remittance Advice" means the electronic (835) or paper explanation of payment that accompanies or follows Payer's adjudication of a claim and states the allowed amount, payment, Member liability, and any adjustment or denial reason codes (CARC/RARC).

1.8 "Utilization Management" means the prospective, concurrent, and retrospective review of the medical necessity, appropriateness, and efficiency of Covered Services, including prior authorization and concurrent review.

ARTICLE 2. SCOPE OF AGREEMENT

2.1 This Agreement governs Provider's participation in Payer's Medicaid Managed Care network and the reimbursement of Covered Services rendered to Members. It applies to all facilities and professional practices listed in Appendix 1 unless a specific facility is expressly excluded by a duly executed Amendment.

2.2 The parties intend that this Agreement be construed as a participating provider agreement and not as a partnership, joint venture, or employment relationship. Each party is an independent contractor and neither party shall have authority to bind the other except as expressly provided herein.

2.3 Provider's exercise of independent medical judgment with respect to the care of any Member shall not be governed or restricted by this Agreement. Nothing herein shall require Provider to render services that Provider determines are not medically appropriate, nor shall any reimbursement provision be construed as an inducement to limit Medically Necessary care.

ARTICLE 3. PROVIDER OBLIGATIONS AND REPRESENTATIONS

3.1 Provider represents and warrants that it and each of its employed and contracted practitioners hold and shall maintain in good standing all licenses, certifications, registrations, and hospital privileges required to render Covered Services in the jurisdiction in which such services are provided.

3.2 Provider shall render Covered Services to Members in the same manner, in accordance with the same standards, and within the same time availability as offered to Provider's other patients, and shall not discriminate against any Member on the basis of source of payment, health status, or any protected classification.

3.3 Provider shall maintain accurate and complete medical, financial, and administrative records relating to Covered Services for a period of not less than ten (10) years, and shall make such records available to Payer and to regulatory authorities as permitted by law and Article 14 (Confidentiality).

3.4 Provider represents that neither it nor any of its practitioners is excluded, debarred, or suspended from participation in any federal or state health care program, and Provider shall promptly notify Payer in writing of any event that would render this representation untrue.

3.5 Provider shall cooperate with Payer's quality improvement, peer review, and member-satisfaction programs and shall furnish encounter and clinical data reasonably necessary for Payer to meet its accreditation and regulatory reporting obligations.

ARTICLE 4. COVERED SERVICES

4.1 Provider shall furnish to Members those Covered Services that fall within Provider's scope of licensure and the categories of service for which Provider is credentialed under Article 6. Covered Services include inpatient hospital services, outpatient and ambulatory services, emergency services, and the professional services of Provider's practitioners.

4.2 Emergency services shall be rendered to any Member presenting with an emergency medical condition without regard to prior authorization, consistent with the prudent-layperson standard and applicable law. Payer shall reimburse Medically Necessary emergency services in accordance with Article 11.

4.3 Services that are excluded from coverage, carved out to a separate vendor, or otherwise not benefits under the applicable Plan are addressed in Article 7. For this Plan, the following carve-out provision applies: Non-emergency transportation and dental carved out to state vendor.

ARTICLE 5. UTILIZATION MANAGEMENT AND MEDICAL NECESSITY

5.1 Payer shall maintain a Utilization Management program that applies objective, clinically based criteria consistent with generally accepted standards of medical practice. Payer shall make its UM criteria available to Provider upon request.

5.2 Coverage determinations that a service is not Medically Necessary shall be made by appropriately licensed clinical personnel, and any adverse determination shall be rendered by a physician or other qualified health professional with expertise in the applicable clinical area. Provider shall have the right to a peer-to-peer discussion prior to the issuance of an adverse determination where reasonably practicable.

5.3 Concurrent review of inpatient admissions shall be conducted in a manner that does not unreasonably interfere with the rendering of Medically Necessary care. Payer shall not retroactively deny an authorized service except in cases of fraud, material misrepresentation, or ineligibility of the Member on the date of service.

ARTICLE 6. CREDENTIALING AND RECREDENTIALING

6.1 Provider and each of its practitioners shall complete Payer's credentialing process prior to rendering Covered Services for which network participation is claimed, and shall be recredentialed not less frequently than once every thirty-six (36) months in accordance with applicable accreditation standards.

6.2 Provider shall submit complete and accurate credentialing applications, including evidence of licensure, board certification, malpractice history, and the insurance coverage required under Article 15. Provider shall notify Payer within five (5) business days of any material change in the credentialing status of any practitioner.

6.3 Payer may suspend or terminate the network participation of any individual practitioner who fails to satisfy credentialing requirements, without thereby terminating this Agreement as to Provider's remaining practitioners and facilities.

ARTICLE 7. EXCLUSIONS AND CARVE-OUTS

7.1 Carve-outs applicable to this Plan: Non-emergency transportation and dental carved out to state vendor.

7.2 Experimental, investigational, and cosmetic services are excluded from coverage and shall not be reimbursed under this Agreement, except where coverage is required by applicable law or an approved clinical trial determination.

7.3 Services rendered to an individual who is not an eligible Member on the date of service, and services that are the financial responsibility of another payer as primary, are excluded from reimbursement under this Agreement and are addressed in Article 8 (Coordination of Benefits).

ARTICLE 8. COORDINATION OF BENEFITS

8.1 Where a Member is covered by more than one health benefit plan, benefits shall be coordinated in accordance with applicable law and the order-of-benefit-determination rules of the National Association of Insurance Commissioners, as adopted by the governing jurisdiction.

8.2 When Payer is the secondary plan, Payer's liability shall not exceed the amount that, when combined with the primary plan's payment, equals the Contracted Rate for the Covered Service. Provider shall pursue primary coverage in good faith and shall furnish primary remittance information with any secondary claim.

8.3 Provider shall identify and bill any liable third party (including workers' compensation, automobile, and liability carriers) as primary where applicable, and shall cooperate with Payer's subrogation and recovery activities.

ARTICLE 9. CLAIMS SUBMISSION AND ADJUDICATION

9.1 Provider shall submit all claims for Covered Services within **95 calendar days** of the date of service (or, for inpatient admissions, the date of discharge). Claims received after this 95-day period may be denied for untimely filing, reported on the Remittance Advice as Claim Adjustment Reason Code (CARC) 29.

9.2 Provider shall submit claims electronically in the ANSI X12 837 format through Payer's designated clearinghouse, using current and valid CPT, HCPCS, ICD-10, revenue, and MS-DRG codes. Paper claims shall be accepted only where electronic submission is not feasible.

9.3 Payer shall adjudicate and remit payment on each Clean Claim within thirty (30) calendar days of receipt via electronic remittance advice (835) and electronic funds transfer (EFT). Interest shall accrue on Clean Claims not paid within the statutory prompt-pay period at the rate required by applicable law.

9.4 Where a claim cannot be adjudicated as a Clean Claim, Payer shall request any additional information in writing within fifteen (15) business days of receipt, and Provider shall respond within a reasonable period. A claim that is denied solely for a curable defect may be corrected and resubmitted within the timely-filing window or thirty (30) days of the denial, whichever is later.

9.5 Payer may recover overpayments by offset against future payments or by written demand, provided that Payer furnishes Provider with an itemized explanation and a reasonable opportunity to contest the recovery before any offset is taken. Recovery of overpayments is subject to the lookback limitations of applicable law.

ARTICLE 10. PRIOR AUTHORIZATION

10.1 Prior authorization is required for elective inpatient admissions, advanced imaging (CT, MRI, PET), and the procedures designated in Exhibit B as requiring prior authorization. Covered Services rendered without a required authorization may be denied, reported on the Remittance Advice as CARC 197 (precertification/authorization absent).

10.2 Provider shall obtain prior authorization in advance of the scheduled service, except for urgent or emergency services, for which authorization (where required) may be obtained within forty-eight (48) hours after the service is initiated. An approved authorization is a determination of Medical Necessity only and does not guarantee Member eligibility or payment.

10.3 Plan-specific authorization and vendor arrangements: Non-emergency transportation and dental carved out to state vendor.

ARTICLE 11. REIMBURSEMENT METHODOLOGY

11.1 Inpatient Covered Services shall be reimbursed at **78% of the then-current Medicare base payment** for the applicable MS-DRG, as set forth in Exhibit A (Inpatient DRG Fee Schedule).

11.2 Outpatient Covered Services shall be reimbursed per the contracted fee schedule in Exhibit B (Outpatient Procedure Fee Schedule). Where no fee schedule rate exists for a Covered Service, payment shall be the lesser of billed charges or the applicable Medicare allowable.

11.3 Payer shall pay the Contracted Rate less applicable Member cost-sharing (deductible, coinsurance, and copayment). The difference between billed charges and the Contracted Rate constitutes a contractual adjustment that Provider shall not bill to the Member (balance billing prohibited), except for applicable Member cost-sharing and non-covered services for which the Member has given informed written consent.

11.4 The rates set forth in the Exhibits are confidential and proprietary, are fixed for the term unless modified by a duly executed Amendment under Article 18, and supersede any conflicting fee schedule previously in effect between the parties.

EXHIBIT A - INPATIENT DRG FEE SCHEDULE

The following contracted case rates apply to inpatient admissions during the term:

DRG	Description	Medicare Base	Factor	Contracted Case Rate
470	Major hip and knee joint replacement w/o MCC	\$14,250	78%	\$11,125
247	Percutaneous cardiovascular proc w/ drug-eluting stent	\$18,900	78%	\$14,750
291	Heart failure and shock w/ MCC	\$9,200	78%	\$7,175
871	Septicemia or severe sepsis w/o MV >96h w/ MCC	\$13,100	78%	\$10,225
190	Chronic obstructive pulmonary disease w/ MCC	\$7,400	78%	\$5,775
194	Simple pneumonia and pleurisy w/ CC	\$6,100	78%	\$4,750
765	Cesarean section w/ CC/MCC	\$8,300	78%	\$6,475
853	Infectious & parasitic dis w/ OR proc w/ MCC	\$31,500	78%	\$24,575
329	Major small & large bowel procedures w/ MCC	\$38,900	78%	\$30,350
064	Intracranial hemorrhage or cerebral infarction w/ MCC	\$12,700	78%	\$9,900

EXHIBIT B - OUTPATIENT PROCEDURE FEE SCHEDULE

CPT	Procedure	Contracted Rate	Prior Auth
27447	Total knee arthroplasty	\$1,255	Yes
70553	MRI brain w/ & w/o contrast	\$375	Yes
45378	Colonoscopy, diagnostic	\$285	No
93452	Left heart catheterization	\$2,300	Yes
99285	Emergency dept visit, high complexity	\$320	No
29881	Knee arthroscopy w/ meniscectomy	\$1,045	Yes

ARTICLE 12. MEMBER GRIEVANCES AND PROVIDER APPEALS

12.1 Provider may appeal a claim determination within **60 calendar days** of the Remittance Advice date. Appeals shall be submitted in writing with supporting clinical and billing documentation and shall identify the claim, the disputed determination, and the basis for the appeal.

12.2 Payer shall acknowledge a provider appeal within fifteen (15) business days and shall render a written determination within thirty (30) calendar days of receipt of a complete appeal. The 60-day filing period applies to first-level appeals; subsequent levels, if any, shall follow Payer's published appeals policy and applicable law.

12.3 Provider shall cooperate with Payer in the resolution of Member grievances relating to Covered Services rendered by Provider, including furnishing relevant medical records and responding to inquiries within the timeframes required by applicable law and accreditation standards.

12.4 Nothing in this Article shall limit a Member's independent right to appeal an adverse benefit determination, to request external review, or to pursue any other remedy available under the Plan or applicable law.

ARTICLE 13. CONFIDENTIALITY AND HIPAA COMPLIANCE

13.1 Each party shall hold in confidence the other party's proprietary and confidential information, including the rates and Exhibits to this Agreement, and shall not disclose such information except as required by law, regulatory authority, or as reasonably necessary to perform its obligations hereunder.

13.2 Each party shall comply with the privacy, security, and breach-notification standards of the Health Insurance Portability and Accountability Act of 1996 (HIPAA), the Health Information Technology for Economic and Clinical Health (HITECH) Act, and their implementing regulations, with respect to all Protected Health Information exchanged under this Agreement.

13.3 The parties shall execute and maintain a Business Associate Agreement where required, and shall implement reasonable administrative, physical, and technical safeguards to protect Protected Health Information. Each party shall report any breach of unsecured Protected Health Information to the other party without unreasonable delay and consistent with applicable law.

ARTICLE 14. INDEMNIFICATION AND INSURANCE

14.1 Each party (the "Indemnifying Party") shall indemnify, defend, and hold harmless the other party and its officers, directors, and employees from and against any third-party claims, damages, and reasonable expenses to the extent arising out of the Indemnifying Party's negligence, willful misconduct, or breach of this Agreement.

14.2 Provider shall maintain, at its sole expense, professional liability (medical malpractice) insurance with limits of not less than \$1,000,000 per occurrence and \$3,000,000 in the aggregate, and commercial general liability insurance in commercially reasonable amounts, with carriers authorized to do business in the applicable jurisdiction.

14.3 Each party shall furnish certificates of insurance upon request and shall provide the other party with not less than thirty (30) days' prior written notice of any cancellation or material reduction in coverage. Self-insurance programs that meet applicable regulatory requirements satisfy this Article.

ARTICLE 15. TERM, TERMINATION AND RENEWAL

15.1 This Agreement is effective January 01, 2024 and continues in full force and effect through December 31, 2025 (the "Initial Term").

15.2 Upon expiration of the Initial Term, this Agreement renews automatically for successive twelve (12) month renewal terms unless either party provides written notice of non-renewal to the other party on or before September 01, 2025.

15.3 Either party may terminate this Agreement for cause upon sixty (60) days prior written notice specifying the alleged breach, provided that the breaching party shall have an opportunity to cure the breach within such sixty (60) day period. Termination shall be effective at the end of the cure period if the breach is not cured.

15.4 Either party may terminate this Agreement without cause upon ninety (90) days prior written notice. Payer may terminate immediately upon Provider's loss of licensure, exclusion from a federal or state health care program, or any event that materially impairs Provider's ability to render Covered Services safely.

15.5 Upon termination, Provider shall continue to render Covered Services to any Member then in an active course of treatment for a transitional period required by applicable law, at the Contracted Rate, to ensure continuity of care. The confidentiality, indemnification, and records-retention obligations survive termination.

ARTICLE 16. GENERAL PROVISIONS

16.1 Governing Law. This Agreement shall be governed by and construed in accordance with the laws of the state in which Provider's principal facility is located, without regard to its conflict-of-laws principles, and subject to applicable federal law.

16.2 Notices. All notices required or permitted under this Agreement shall be in writing and shall be deemed given when delivered personally, sent by nationally recognized overnight courier, or sent by certified mail, return receipt requested, to the addresses set forth in Appendix 1 or as otherwise designated in writing by a party.

16.3 Assignment. Neither party may assign this Agreement, in whole or in part, without the prior written consent of the other party, except that either party may assign to an affiliate or successor in connection with a merger or sale of substantially all of its assets, upon written notice to the other party.

16.4 Amendment. Except as otherwise expressly provided herein, this Agreement may be amended only by a written instrument signed by authorized representatives of both parties. Amendments to the Exhibits and Fee Schedules are governed by Article 18 of any applicable Amendment and take effect on the stated effective date.

16.5 Force Majeure. Neither party shall be liable for any failure or delay in performance to the extent caused by events beyond its reasonable control, including acts of God, natural disaster, public health emergency, war, terrorism, labor disturbance, or failure of utilities or communications, provided that the affected party uses reasonable efforts to resume performance.

16.6 Entire Agreement. This Agreement, together with its Exhibits, Appendices, and duly executed Amendments, constitutes the entire agreement between the parties with respect to its subject matter and supersedes all prior understandings, whether oral or written.

16.7 Severability. If any provision of this Agreement is held to be invalid or unenforceable, the remaining provisions shall continue in full force and effect, and the parties shall negotiate in good faith a valid provision that most nearly effects the original intent.

16.8 Waiver; Counterparts. No waiver of any provision shall be effective unless in writing, and no waiver shall constitute a continuing waiver. This Agreement may be executed in counterparts, including by

electronic signature, each of which shall be deemed an original and all of which together constitute one instrument.

IN WITNESS WHEREOF, the parties have caused this Agreement to be executed by their duly authorized representatives as of the Effective Date first written above.

PAYER:

Pioneer Valley Medicaid MCO

By: _____

Name: VP, Network Management

Title: Vice President

Date: January 01, 2024

PROVIDER:

Northwind Health

By: _____

Name: SVP, Revenue Cycle

Title: Senior Vice President

Date: January 01, 2024